Provider Enablement Intel

> **Week of November 2, 2026**
> Narrative created: November 2, 2026

Weekly strategy brief on Medicare Advantage and Medicaid risk-contracting economics, payer partners, and value-based care enablement platforms.

In the News

- 1. California removes 1.9 million members from Medicaid managed-care risk
- 2. Privia buys attributed lives and a pathway into physician practices
- 3. Centene's MA margin recovery now runs ahead of its 2028 Stars economics

Executive View

- California's January 1 conversion of about 1.9 million Medi-Cal members from managed care to fee-for-service will remove a large population from capitated contracts; Centene estimates that approximately 175,000 at-risk Health Net members are affected. (California Health Care Foundation, *Medi-Cal Enrollees with Unsatisfactory Immigration Status: What to Know About the Transition to Fee-for-Service*, October 30, 2026; Centene, *Form 10-Q for Quarter Ended September 30, 2026*, October 27, 2026)
- Privia paid approximately $36 million for a Cano Health ACO with more than 36,000 MSSP lives, expanding its attributed-life base for less than $1,000 per starting beneficiary without acquiring the underlying practices. (Privia Health, *Privia Health Expands VBC Footprint with Acquisition of American Choice Healthcare MSSP Business*, November 2, 2026)
- Centene's Medicare operations improved faster than planned, but the share of its MA membership in four-star contracts fell from approximately 20% to 10%, creating a quality-bonus headwind for 2028 revenue. (Centene, *Third Quarter 2026 Results*, October 27, 2026; Centene, *Form 10-Q for Quarter Ended September 30, 2026*, October 27, 2026)
- Overall environment: deteriorated modestly because California's removal of 1.9 million members from managed-care risk outweighs a small ACO acquisition and Centene's near-term

MA margin recovery.

1. California removes 1.9 million members from Medicaid managed-care risk

Status: NEW

What happened

An October 30 issue brief quantified California's January 1, 2027 transition of Medi-Cal members with unsatisfactory immigration status from managed care to fee-for-service at approximately 1.9 million people, including 251,000 children. It found that 58.7% of affected members receiving active treatment use providers that are not enrolled in Medi-Cal fee-for-service. The state has confirmed that affected members retain coverage but lose access to managed-care-only Enhanced Care Management and Community Supports. (California Health Care Foundation, *Medi-Cal Enrollees with Unsatisfactory Immigration Status: What to Know About the Transition to Fee-for-Service*, October 30, 2026; California DHCS, *Transition to Medi-Cal Fee-for-Service*, accessed November 2, 2026)

Centene separately disclosed that approximately 175,000 at-risk Health Net members will leave its Medi-Cal contracts, reducing membership and premium revenue beginning in 2027. (Centene, *Form 10-Q for Quarter Ended September 30, 2026*, October 27, 2026)

Implication

The conversion removes capitation and care-management funding rather than coverage. Risk-bearing provider groups lose attributed Medicaid lives and must replace predictable managed-care payments with state claims submission, authorization and cash-collection workflows. Near-term enablement demand shifts toward fee-for-service enrollment, eligibility reconciliation and continuity-of-care operations, while demand for plan-funded quality and population-management services declines for this population.

Watch next

DHCS guidance on existing authorizations and complex-care transitions, provider fee-for-service enrollment rates, and whether plans amend downstream capitation before January 1.

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2. Privia buys attributed lives and a pathway into physician practices

Status: NEW

What happened

Privia acquired American Choice Healthcare MSSP, a Cano Health subsidiary whose ACO will manage more than 36,000 MSSP-attributed lives beginning January 1, 2027. Privia paid approximately $36 million in cash at closing and expects minimal 2026 impact but positive contributions to attributed lives and adjusted EBITDA in 2027. The ACO includes practices in existing and new Privia states. (Privia Health, *Privia Health Expands VBC Footprint with Acquisition of American Choice Healthcare MSSP Business*, November 2, 2026)

Implication

Privia is acquiring an accountable-care contract and physician relationships for less than $1,000 per starting attributed life, rather than buying or employing the practices. That preserves its lower-capital affiliation model and gives Privia Care Partners an immediate population-health revenue base plus a channel for selling practices the broader medical-group, technology and revenue-cycle platform.

The economics depend on beneficiary attribution, provider retention and the acquired ACO's benchmark position, none of which Privia disclosed. The structure nevertheless establishes a replicable corporate-development route for consolidating independent ACO assets as smaller operators or legacy primary-care platforms rationalize portfolios.

Watch next

Privia's November 5 disclosure of the ACO's geographic mix, expected shared-savings contribution, physician retention terms and integration costs.

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3. Centene's MA margin recovery now runs ahead of its 2028 Stars economics

Status: UPDATE

What happened

Centene's Medicare segment, which includes MA, Part D and Medicare Supplement, recorded a third-quarter health-benefits ratio of 93.8%, down from 94.3% a year earlier. Management said the MA business is on course to reach breakeven in 2026, one year earlier than its multiyear turnaround

plan. (Centene, *Third Quarter 2026 Results*, October 27, 2026; The Motley Fool, *Centene Q3 2026 Earnings Call Transcript*, October 27, 2026)

The quality outlook moved in the opposite direction. Centene disclosed that approximately 10% of its MA membership is in four-star contracts under the 2027 ratings, down from 20% under the prior ratings; 42% is in contracts rated at least 3.5 stars, versus 60% previously. The new ratings affect 2028 revenue. (Centene, *Form 10-Q for Quarter Ended September 30, 2026*, October 27, 2026)

Implication

Pricing and medical-cost execution reduce the immediate risk of Centene retrenching further from MA provider relationships. The lower four-star exposure, however, creates pressure to recover quality-bonus revenue through care-gap closure, medication adherence, member experience and tighter provider performance terms. The enablement opportunity therefore shifts from turnaround support toward quality remediation ahead of the 2028 payment year.

Watch next

Centene's quantification of the 2028 revenue effect and whether 2027 provider contracts place more compensation at risk for Stars measures.

What to Watch For

- November 4, 2026: agilon reports third-quarter results, with medical margin, payer repricing and MA membership retention as the key tests. (agilon health, *agilon health Sets Date to Report Third Quarter 2026 Financial Results*, October 5, 2026)
- November 5, 2026: Privia, Astrana and Evolent report results, providing updates on acquired ACO integration, full-risk cost performance and specialty-management customer membership. (Privia Health, *Privia Health to Report Third Quarter 2026 Results*, October 8, 2026; Astrana Health, *Astrana Health Schedules 2026 Third Quarter Financial Results*, October 9, 2026; Evolent Health, *Evolent to Release Third Quarter 2026 Financial Results*, October 9, 2026)
- November 6, 2026: Humana reports third-quarter results, including the first detailed view of 2027 MA enrollment expectations and CenterWell risk economics. (Humana, *Humana Inc. to Release Third Quarter 2026 Results on November 6, 2026*, October 12, 2026)
- November 9, 2026: P3 Health reports third-quarter results following its payer and provider-network rationalization. (P3 Health Partners, *P3 Health Partners Schedules Third Quarter 2026 Earnings Release and Conference Call*, October 22, 2026)
- January 1, 2027: California transitions affected Medi-Cal members from managed care to fee-for-service. (California DHCS, *Transition to Medi-Cal Fee-for-Service*, accessed November 2,

2026)

Bottom Line

California's removal of 1.9 million members from managed-care risk is the week's most consequential change, with Health Net-contracted providers and groups lacking Medi-Cal fee-for-service enrollment requiring the closest attention. Privia's acquisition shows that attractive ACO assets can still be added without employing physicians. The November 4-6 earnings cluster is the next catalyst most likely to change the view on full-risk medical-cost performance and 2027 payer contracting.